Infection prevention and control and water, sanitation and hygiene in health emergency readiness for response operations capabilities

User guide and checklist

World Health Organization

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Acronyms

HCFs	Health-care facilities
IPC & WASH HERO CAPE	Infection prevention and control and water, sanitation and Hygiene in health emergency readiness for response operations capabilities
IPC	infection prevention and control
SOP	SOP Standard operating procedure
WASH	water, sanitation and hygiene

Glossary

The following definitions were provided for words and phrases found in the text and relate to their use in this checklist's context only.

Capability level	indicates how far the country has progressed towards attaining a given indicator, component and core capacity *(1)*.
Coordination mechanism	Mechanism /body is available and functional, with terms of reference, membership from all relevant sectors, established communications channels, access to decision-makers and contacts, joint activities, meeting reports, plans, and evaluation *(1)*.
Designated health facilities	refers to priority health facilities (HFs) that are located in high-risk areas and are expected to deal with priority public health threats.
Health-care facilities	encompass all formally recognized facilities that provide health-care services, including primary (health posts and clinics); secondary and tertiary (district or national hospitals); public and private (including faith-run); and temporary structures designed for emergency contexts (e.g. cholera treatment centers). They may be located in urban or rural areas *(2)*.
Operational readiness	the outcomes of preparedness actions, including planning, allocation of resources, training, exercising and organizing to build, sustain and improve operational capabilities based on risk assessments (WHO Emergency Response Framework). the ability to quickly and appropriately respond when required (UN General Assembly) *(4)*.
Readiness	The state which links effective preparedness to efficient relief; a statement of the capacity and capability of a relief agency or service. These activities may take up to 6 months in order to ensure readiness for a specific defined threat *(3)*.
Readiness capabilities	Encompass not only the physical or quantitative aspects of a system to respond to an emergency, but also, the knowledge, skills, expertise, and processes an organization possesses to be ready to respond to imminent threats *(5)*. refers to the capability of a system or technical abilities needed to achieve and maintain operational readiness (e.g. the status of a coordination mechanism, assessment system, etc.).
Readiness sub-capabilities	refers to the status/conditions of specific technical areas necessary for achievement and maintenance of their capability (e.g. specific tools, guidance, personnel, logistics and supplies, etc., relevant to maintain/ achieve the capability) *(5)*.
References and tools	incorporates relevant technical guidance, templates, tools and explanatory notes about each capability that helps the users better understand and adopt them.
Response	is the stage in which emergency actions in response to a defined public health threat exceed the usual level of activities *(3)*.
Responsive	Ability to respond quickly and positively

Subnational	refers to any government entity below the national level, regardless of the political, financial and administrative design of the country *(3)*.
Surveillance	The systematic ongoing collection, collation and analysis of data for public health purposes and the timely dissemination of public health information for assessment and public health response, as necessary *(4)*.
Verification	Provision of information by a State Party to WHO confirming the status of an event within the territory or territories of that State Party *(4)*.
Verifiers	refers to supporting documents related to each assessed item, or criteria for direct observation further described in technical guidance.

Introduction

The Infection prevention and control and water, sanitation and Hygiene in health emergency readiness for response operations capabilities checklist, herein referred to as the IPC & WASH HERO CAPE is a rapid self-assessment tool designed to help health officials evaluate the key functionalities of IPC and WASH readiness capabilities and sub-capabilities necessary for response to priority public health threat(s).

This assessment aims to strengthen national and subnational infection prevention and control (IPC) and water sanitation and hygiene (WASH) operational readiness for responding to a public health threat or infectious disease outbreaks by evaluating existing capabilities, identifying gaps and guiding strategic improvements. Its structure was based on WHO-Framework and toolkit for infection prevention and control in outbreak preparedness, readiness and response at the national level the user of this checklist should be familiar with the framework's contents. The IPC & WASH HERO CAPE ensures a standardized approach to enhancing IPC and WASH measures by promoting effective response strategies and fostering resilience against public health emergencies.

Description of the tool

The IPC & WASH HERO CAPE is a structured, closed-format checklist with an associated scoring system. This user guide provides instructions for completing the IPC & WASH HERO cape, which is designed in Microsoft Excel, it includes five worksheets (definitions, introduction, user guide, tool to calculate the score and improvement plan). The IPC & WASH HERO CAPE checklist is intended to be primarily self-administered (a self-assessment tool) to capture the functionality of six critical readiness capabilities and 23 sub-capabilities necessary to mount an effective IPC and WASH response to priority public health threat(s) or outbreaks. This tool assesses country-level readiness towards priority public health threat(s), rather than readiness at the level of health facilities.

This checklist was adopted from the HERO CAPE, a tool informed by lessons learned from the experiences of WHO regional and country health emergency experts during public health emergencies. The HERO CAPE tool includes feedback from consultations with health emergency experts in fragile, conflict-affected and vulnerable settings. The IPC & WASH HERO CAPE tool builds upon this foundation by aligning with relevant frameworks and International Health Regulations (IHR) indicators, including:

- Framework and toolkit for infection prevention and control in outbreak preparedness, readiness and response at the national: https://iris.who.int/handle/10665/345251 *(3)*.
- Emergency response framework (ERF), Edition 2.1:https://www.who.int/publications/i/item/9789240058064 *(5)*.
- Health emergency prevention, preparedness, response and resilience (HEPR) global Architecture – safe and scalable care - 3.2 – Protection of health workers and patients : https://www.who.int/publications/m/item/strengthening-the-global-architecture-for-health-emergency-prevention--preparedness--response-and-resilience *(6)*.
- Joint external evaluation tool: International Health Regulations (2005), third edition – respond R4 – Infection prevention and control capacities and technical questions *(7)*.
- States parties' self-assessment annual reporting tool, 2nd edition – capacity C9 - Infection prevention and control capacities *(8)*.

Who should use this checklist (method)?

Upon identification of a priority public health or communicable disease threat (e.g. Ebola virus disease [EBOD], cholera, COVID-19, Lassa fever, multidrug-resistant organisms, influenza, etc.) and, upon review of the available local, regional and global epidemiological data, the national team will:

1. **Identify and assign the priority health facilities (HFs)**: When deciding which national HFs to designate as priority HFs to address identified priority public health threats, it is advisable to ensure that the number of designated facilities is proportionate to the overall number of facilities. This should be based on geographical distribution of HFs, the population at risk and the context of the diseases.
2. **Assess the IPC and WASH readiness at the designated HFs**: using appropriate health-care facilities' assessment tools, which could include but are not limited to:
   - *The infection prevention and control minimum requirements assessment tools for primary, secondary, or tertiary health-care facilities (*(Target is to achieve is 100% of the overall score) *(9,10,11).*
   - *The water, sanitation, and hygiene facility improvement tool, 2nd edition (WASH FIT) (Target is to achieve ≥80% of the overall score) (12).*
   - The disease-specific, rapid assessment tools, for example (COVID-19, cholera, mpox *(13)*., EBOD *(14)*.) (Target is to achieve ≥80% of the overall score)
   - National IPC assessment tools for health-care settings *(Target is to achieve ≥80% of the overall score)*
3. **Assess country capabilities**: Use the user guide to assess the capabilities mentioned in the tool. Each capability is divided into sub-capabilities, which are assessed and given a score (1-5) using a traffic light scheme (as shown below). The evaluation scores of the capabilities and the overall readiness score are calculated automatically, resulting in a percentage score. There is also a field for comments or notes and a field for potential verifiers.
4. **Develop and submit an improvement plan**: After completing the assessment and identifying gaps/areas for improvement, develop an improvement plan that contains recommended actions and responsibilities. This plan should be submitted to decision-makers after discussions with all concerned parties.
5. **Follow-up assessment**: it is advisable to conduct a follow-up assessment after the baseline and update the progress in the improvement plan every 6 months or whenever there is change in the situation.
6. **Use in conjunction with national plans**: This checklist should also be used in conjunction with national strategic preparedness and response plans and/or contingency plans for specific diseases or priority public health threats.

Traffic light scoring

When scoring the checklist*. The scores have been designed to follow a traffic light system, as outlined in Table 1.

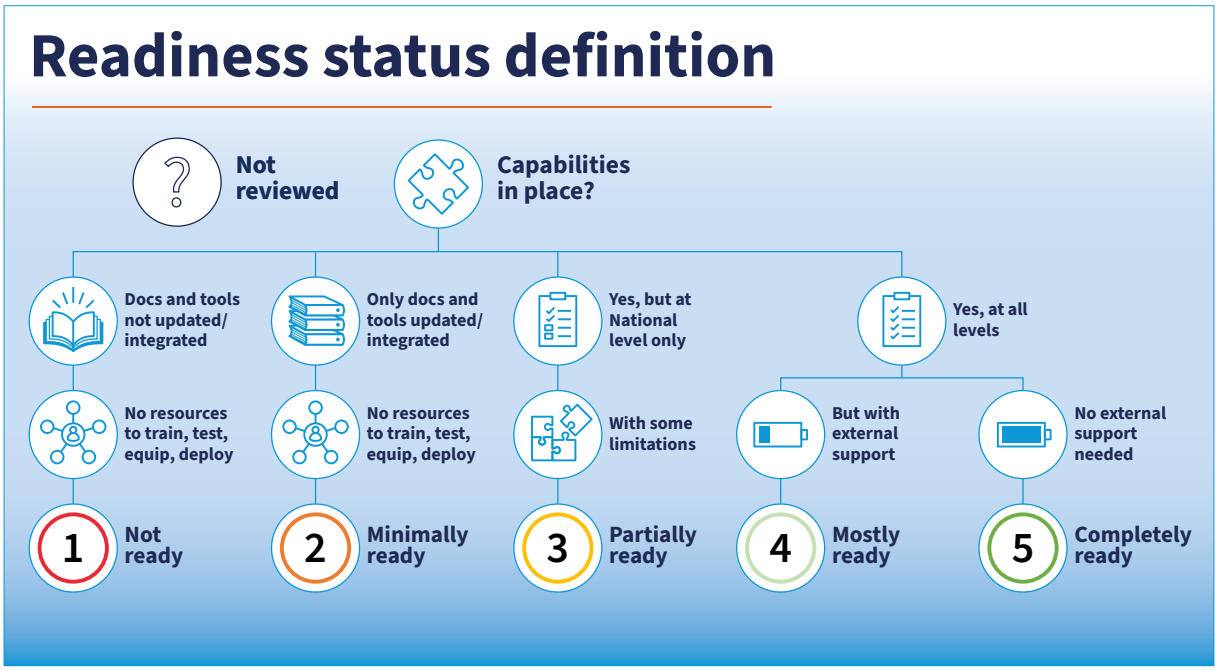
Table 1. Traffic light scoring

Not ready	
Score "1" (0-20%)	The sub-capabilities are not in place (e.g. no/few updated guides, protocols or strategies/tools).
Minimally ready	
Score "2" (20-40%)	The sub-capabilities are under development (e.g. documentation work has been updated but no resources are in place for implementation, e.g. no trainings/SimEx have been conducted, no financial resources and workforces are ready to be mobilized).
Partially Ready	
Score "3" (40-60%)	The sub-capabilities are at national level only, with some limitations meeting the demands (updated documentations/ protocols, tools, plans, SOPs and resources are in place).
Mostly Ready	
Score "4" (60-80%)	The sub-capabilities are in place at national and subnational levels and/or among some of the designated health facilities[1] (but need external technical, workforce and financial support).
Completely Ready	
Score "5" (80-100%)	The sub-capabilities are in place at national and subnational levels and/or among all of the designated HFs (no need for external support; the country is able to manage all the resources and technical capabilities by internal capacity only).

*Except for the assessment of the sub-capabilities 5.3 and 5.4

The numerator represents the number of health facilities that have achieved a target score of 80% or higher, except in cases where the IPC minimum requirement is 100%. The denominator is the total number of designated health facilities assigned by the country.

Interpretation of readiness or response status



Why use this tool?

By scoring each capability and the overall performance, we identify strengths and achievements as well as areas for improvement. Identifying strengths boosts confidence and assures decision-makers that success is achievable. Recognizing gaps highlights key areas that need additional attention and creates a sense of urgency for making necessary improvements. The checklist serves as an indicator of the country's IPC and WASH readiness for public health emergencies. These results can guide an improvement plan to enhance measures and drive efforts at both national and sub-national levels. Regular updates enable progress to be tracked over time.

Feedback from users and further develompent of the tool

Comments and suggestions from IPC & WASH-HERO users will be collected, which will help us to revise and update the tools regularly. Minor changes will be reflected in the Excel documents and major updates will be provided both in the Excel files and in the printed versions from time to time.

User guide

Capability 1. IPC and WASH coordination mechanism(s) is/are in place at the national, subnational and designated priority HF levels

Sub-capabilities	Scores		Resources and tools:
1.1. There is a coordination mechanism in place that provides oversight of the IPC and WASH strategy, technical and operational actions and activities for the priority public health threats (e.g. national IPC committee, IPC task force or technical working groups (TWGs)) supported (e.g. co-chaired) by WHO and including all relevant stakeholders and implementing partners, such as national/ international NGOs and health-care facilities' leaders.	1	The coordination mechanism is not in place.	• Global strategy on infection prevention and control • Framework and toolkit for infection prevention and control in outbreak preparedness, readiness and response at the national level • Emergency response framework EFR, edition 2.1 • Incident command and management system: public health guidance for community-level preparedness and response to severe acute respiratory syndrome (SARS). Atlanta: Centers for Disease Prevention and Control
	2	The coordination mechanism is under development, e.g. TOR have been drafted, but no resources are available.	
	3	The coordination mechanism has been activated/established at the national level only, with limited resources available.	
	4	There is a coordination mechanism at the national and subnational levels, but they need external technical, financial support.	
	5	There is a coordination mechanism at the three levels (national, subnational and all designated HFs) and it is managed without external support.	
1.2. There is a communication strategy in place that has been adapted from and integrated with the broader, existing one, to disseminate IPC- and WASH-related information on the priority public health threat or outbreak to the stakeholders and various groups (HFs, health and care workers, community and public social media) using available platforms (social media, official website, WhatsApp groups, contact list emails, hotlines, etc.) in coordination with partners in risk communication and community engagement (RCCE).	1	A communication strategy is not in place.	• WHO outbreak communication planning guide
	2	A communication strategy and platform(s) is/are under development.	• Outbreak communication planning guide.
	3	A communication strategy and platform is in place but only at the national level and there is no evidence of dissemination or coordination with the RCCE team.	• Communicating risk in public health emergencies: a WHO guideline for emergency risk communication (ERC)policy and practice
	4	A communication strategy, platform and coordination mechanism are in place, but there is a need for external support (technical, workforce, financial).	• Communication for behavioral impact (COMBI) - toolkit

Sub-capabilities	Scores		Resources and tools:
	5	A communication strategy, platform and coordination mechanism are in place and the country is managing it without any external support.	-
1.3. There is an IPC partners' mapping mechanism in place using "5 Ws" to prepare and coordinate the IPC readiness activities for the priority public health threat(s).	1	An IPC partners' mapping mechanism is not in place.	• IPC Outbreak Partners mapping template
	2	An IPC partners' mapping mechanism is under development.	
	3	An IPC partners' mapping mechanism is in place, but only at the national level and there is no evidence of coordination.	
	4	An IPC partners' mapping mechanism is in place at national and subnational levels, but there is a need for external support (technical, human resources, financial).	
	5	An IPC partners' mapping mechanism is in place at national and subnational levels and at all designated health facilities (HFs), and it is being managed without need for external support.	
1.4. There is a functioning IPC stakeholder network in place at the national and subnational levels and among the designated HFs to handle priority public health threat(s). For example, IPC national focal points (FPs), quality and patient safety FPs, IPC leaders at the health facility level, hospital administrators and key decision-makers at the national and subnational levels for readiness and response to the identified priority public health threat(s).	1	The IPC stakeholder network is not in place or not functioning.	• Stakeholders mapping template
	2	The IPC stakeholder network is under development.	
	3	An IPC stakeholder network is in place, but only at the national level.	
	4	An IPC stakeholder network is in place at the national and subnational levels that includes some leaders of the designated HFs.	
	5	An IPC stakeholder network is in place at the three levels (the national, subnational and among all of the leaders of the designated HFs).	

Capability 2. IPC and WASH readiness plans needed for the priority public health threat(s) are in place

Sub-capabilities	Scores		Resources and tools:
2.1. There is a national contingency plan for specific priority public health threat(s) that includes IPC and WASH readiness activities with an allocated budget.	1	No national contingency plans for specific priority public health threat(s) exist that include IPC and WASH or, if they do exist, they are outdated.	• WHO guidance for contingency planning • IPC outbreak contingency plan
	2	The national contingency plans for specific priority public health threat(s), including IPC and WASH, are under development or in the process of being updated.	
	3	The national contingency plans for specific priority public health threat(s), including IPC and WASH, are at the national level only.	
	4	The national contingency plans for specific priority public health threat(s), including IPC and WASH, are at the national and subnational levels and have been disseminated to some of the designated HFs and there is an identified need for external technical, workforce and financial support.	
	5	The national contingency plans for specific priority public health threat(s), including IPC and WASH, are at the national and subnational levels and have been disseminated to all of the designated HFs and the country is managing them without any external support.	
2.2. There is a surge-capacity plan for the priority threat with allocated resources for IPC and WASH activities/interventions, including staffing, infrastructure and supplies.	1	No surge capacity plans for specific priority public health threat(s) exist or, if they do, they do not include IPC and WASH or they are outdated.	• Adaptt Surge Planning Support Tool
	2	The surge capacity plan for specific priority public health threat(s), including IPC and WASH, are under development or are in the process of being updated.	

Sub-capabilities	Scores		Resources and tools:
	3	There is a surge capacity plan for specific priority public health threat(s) that includes IPC and WASH, but only at the national level, not at the subnational level nor among the designated HFs.	
	4	There is a surge capacity plan for specific priority public health threat(s) that includes IPC and WASH at the national and subnational levels and among some of the designated HFs, but there is an identified need for external technical, workforce and financial support.	
	5	There is a surge capacity plan for specific priority public health threat(s) that includes IPC and WASH at the national and subnational levels and that has been disseminated among all of the designated HFs and the country is managing it without any external support.	
2.3.There is a responsive stockpiling process in place to ensure adequate IPC and WASH supplies and equipment.	1	The stockpiling process is not in place, does not exist or does not include IPC and/or WASH.	• WHO COVID-19 Essential Supplies Forecasting Tool (COVID-ESFT) v4.1
	2	The stockpiling process is under development.	
	3	There is a stockpiling process at the national level only.	
	4	There is a stockpiling process at the national and subnational levels that includes some of the designated HFs, but it requires external support.	
	5	There is a stockpiling process at the national and subnational levels that includes all of the designated HFs and the country manages all needed resources (trained staff, tools, logistics and financial support).	
2.4.There is a mechanism in place to identify the priority HFs ensuring the coordination, delivery of resources (e.g. supplies) and an isolation-referral system in the event of a surge.	1	The mechanism is not in place or has not yet identified the priority health facilities.	-

Sub-capabilities	Scores		Resources and tools:
	2	The mechanism is under development, with no available resources to continue.	
	3	The mechanism is in place at the national level only, with limited resources.	
	4	The mechanism is in place at the national and subnational levels and for some of the designated HFs.	
	5	The mechanism is in place at the national and subnational levels and for all of the designated HFs.	

Capability 3. IPC and WASH guidelines, policies, SOPs and/or standards for priority public health threat(s) are in place at the national level and in the designated priority HFs

Sub-capabilities	Scores		Resources and tools:
3.1. There are national IPC guidelines, policies and/or SOPs for the priority public health threat(s).	1	No IPC guidelines, policies or SOPs for the priority public health threat(s) are in place or they are outdated.	• Health Emergencies - Infection prevention and control and water sanitation and hygiene publications
	2	The IPC guidelines, policies or SOPs for the priority public health threat(s) are under development or in the process of being updated.	• WHO Infection prevention and control and water, sanitation and hygiene measures for diphtheria in health-care settings: operational guide
	3	There are updated IPC guidelines, policies and SOPs for the priority public health threat(s), but they are at the national level only.	• WHO Infection prevention and control and water, sanitation and hygiene measures for Crimean-Congo haemorrhagic fever in health-care settings: operational guide
	4	There are updated IPC guidelines, policies and SOPs for the priority public health threat(s) at the national and subnational levels and among some of the designated HFs, but there is an identified need for external support (e.g. technical, workforce and financial).	• WHO Infection prevention and control and water, sanitation and hygiene measures in health-care settings and shelters/ congregate settings in Gaza: Technical note, 22 February 2024
	5	There are updated IPC guidelines, policies and SOPs for the priority public health threat(s) at the national and subnational levels and among all of the designated HFs and the country is managing them without any external support.	
3.2. There are national WASH standards for the priority public health threat(s).	1	No WASH national standards for the priority public health threat(s) are in place or they are outdated.	• WHO Infection prevention and control guideline for Ebola and Marburg disease, August 2023
	2	The WASH national standards for the priority public health threat(s) are under development or in the process of being updated.	

Sub-capabilities	Scores		Resources and tools:
	3	The WASH national standards for the priority public health threat(s) are at the national level only.	-
	4	The WASH national standards for the priority public health threat(s) are at the national and subnational levels and among some of the designated HFs, but there is an identified need for external technical, workforce and financial support.	-
	5	The WASH national standards for the priority public health threat(s) are at the national and subnational levels and among all of the designated HFs and the country is managing them without any external support.	-
3.3. There is a dissemination plan for the national IPC and WASH guidelines, policies SOPs and/or standards on the priority public health threat(s) on the national/ subnational levels (e.g. MOH, National-level CDC) to the designated HF(s).	1	No system in place.	
	2	The system is in process of development.	-
	3	There is a system for dissemination at the national level, but it has not yet been used.	-
	4	The national IPC and WASH plans, guidelines and/or SOPs have been disseminated to subnational level and to some of the designated HFs.	-
	5	The national IPC and WASH plans, guidelines and/or SOPs have been disseminated to subnational level and to all of the designated HFs.	

Capability 4. IPC and WASH training programmes on the priority public health threat(s) are in place at the national, subnational and designated HF levels

Sub-capabilities	Scores		Resources and tools:
4.1. The IPC and WASH training curriculum and materials on priority public health threat(s) have been developed or adapted from the national guidelines.	1	No IPC and or WASH training materials exist, or they exist but are outdated or not tailored to the priority public health threat(s).	• Open WHO
	2	The IPC and/or WASH materials are under development or in process of being adopted or updated.	• Core competencies for infection prevention and control professionals. G
	3	The IPC and/or WASH materials are at the national level only.	• Rapid Response Teams Training Programme programme
	4	The IPC and or WASH materials are at the national and subnational levels and have been disseminated to some of the designated HFs.	
	5	The IPC and or WASH materials are at the national and subnational levels and have been disseminated to all of the designated HFs.	
4.2. There is a national and/or subnational IPC and WASH training programme that actively provides training on priority public health threats.	1	No training programmes are in place.	
	2	The training programmes are under development.	
	3	There is a training programmes are at the national level only, but with limited resources in place to implement it.	
	4	There are training programmes at the national and subnational levels and among some of the designated HFs and there is an identified need for external support (staffing, logistics and/or financial) to implement it completely.	
	5	The training programmes are at the national and subnational levels and among all of the designated HFs and the country manages them without any external support (staffing, logistics and/or financial).	

Sub-capabilities	Scores		Resources and tools:
4.3. There are IPC and WASH information, education and communication (IEC) materials developed that communicate relevant messages for health and care workers, patients, visitors and the general population on the identified priority public health threat(s) and it has been disseminated to the designated HFs and to the community.	1	There are no IEC materials.	
	2	The materials are under development.	
	3	The IEC materials exist at the national level only and have not been disseminated to the subnational level nor to the designated HFs.	
	4	The IEC materials are at the national level and have been disseminated to the subnational level and some of the designated HFs and there is an identified need for external support (staffing, logistics and or financial).	
	5	The IEC materials are at the national level and have been disseminated to the subnational level and to all of the designated HFs without need for external support (staffing, logistics and/or financial).	

Capability 5. IPC and WASH assessments at the national, sub-national and the designated HFs to evaluate the readiness to priority public health threat(s)

Sub-capabilities	Scores		Resources and tools:
5.1. There is system in place that includes all the resources required (trained staff, tools, logistical and financial support) to conduct the IPC and WASH assessment at the designated HFs.	1	No system is in place.	• Infection prevention and control assessment framework at the facility level IPCAF
	2	The system is under development, but no resources are in place for implementation.	• Assessment tool on infection prevention and control minimum requirements for primary, secondary and tertiary health care facilities
	3	The system is in place at the national level only in terms of the trained person(s), logistics and tools.	• Assessment tool on infection prevention and control minimum requirements for primary health care facilities
	4	The system is in place at the national and subnational levels and among some of the designated HFs with an identified need for external support (trained staff, tools, logistics and financial support).	• Assessment tool on infection prevention and control minimum requirements for secondary health care facilities
	5	The system in place at the national and subnational levels and among all of the designated HFs; the country manages all needed resources (trained staff, tools, logistics and financial support).	• Assessment tool on infection prevention and control minimum requirements for tertiary health care facilities
5.2. IPC and WASH simulation exercises are conducted to test the response to the priority threats or are scheduled to be completed within six months from the time that the 5.3. priority public health threat(s) has been identified.	1	No simulations, drills and/or tabletop exercises have been conducted or scheduled.	• WASH FIT: A practical guide for improving quality of care through water, sanitation and hygiene in health care facilities. Second edition
	2	Simulations, drills and/or tabletop exercises are under development.	• Rapid hospital readiness checklist for COVID-19
	3	Simulations, drills and/or tabletop exercises have been conducted at the national level only.	-
	4	Simulations, drills and/or tabletop exercises have been conducted at the national and subnational levels and among some of the designated HFs.	-

Sub-capabilities	Scores		Resources and tools:
	5	Simulations, drills and/or tabletop exercises have been conducted at the national and subnational levels and among all of the designated HFs.	-
5.4. The IPC assessments have been completed in designated HFs using the appropriate IPC assessment tools for the priority public health threat(s).	1	No IPC assessment has been conducted yet or only 20% or fewer of the designated facilities achieved the target score.	-
	2	The IPC assessment has been completed in 40% of the designated HFs that achieved the target score.	-
	3	The IPC assessment has been completed in 60% of the designated HFs that achieved the target score.	-
	4	The IPC assessment has been completed in 80% of the designated HFs that achieved the target score.	-
	5	The IPC assessment has been completed in 100% of the designated HFs that achieved the target score.	-
5.5. The WASH assessments have been completed in designated HFs using WASH assessment tools appropriate to the priority public health threat(s), e.g. WASH-FIT tool.	1	No WASH assessment has been conducted yet or the target score was achieved in 20% or fewer of the designated HFs.	-
	2	The WASH assessment has been completed in 40% of the designated HFs that achieved the target score.	-
	3	The WASH assessment has been completed in 60% of the designated HFs that achieved the target score.	-
	4	The WASH assessment has been completed in 80% of the designated HFs that achieved the target score.	-
	5	The WASH assessment has been completed in 100% of the designated HFs that achieved the target score.	

Capability 6. National surveillance and reporting programme for early detection of the priority public health threat(s), especially among hospitalized patients and health and care workers

Sub-capabilities	Scores		Resources and tools:
6.1. Active coordination with the national and subnational surveillance networks to include syndromic and microbiologic surveillance for the priority public health threat(s) is in place.	1	No active surveillance networks are in place.	• Surveillance of health care-associated infections at national and facility levels
	2	A surveillance network is under development, drafted TOR, with no available resource to implement.	• IPC outbreak surveillance
	3	The surveillance network has been activated/established at the national level only, not at the subnational level nor at the level of the designated priority HFs with limited resources.	
	4	There is a surveillance network at the two levels (national and subnational), but they need external technical, financial support.	
	5	There is a surveillance network at the three levels (national, subnational and among the designated HFs), and they do not need external support.	
6.2. There is a plan and mechanism in place to implement reporting and data collection on the infected patients and health and care workers that includes the occupational management of health and care worker infections.	1	A plan and mechanism for data collection and reporting is not in place.	
	2	A plan and mechanism for data collection and reporting is under development, TOR have been drafted, but no resources are available to implement.	
	3	A plan and mechanism for data collection and reporting has been activated/established at the national and subnational levels, but resources available to implement them are limited.	

Sub-capabilities	Scores		Resources and tools:
	4	A plan and mechanism for data collection and reporting at the three levels (national and subnational levels and among some of designated HFs) has been activated/established, but reporting data on patients, health and care workers and some of the designated HFs need external technical and financial support.	
	5	The plan and mechanism for data collection and reporting at the three levels (national and subnational and among all of the designated HFs) has been activated/established on both the patients and health and care workers without need for external support.	
6.3. The surveillance tools for priority public health threa(s) with standardized case definitions are in place and have been disseminated to the designated HFs.	1	The surveillance tools are not in place.	
	2	The surveillance tools are under development.	
	3	The surveillance tools are available at the national level only and have not been disseminated at the subnational level nor among the designated priority HFs, whose resources are limited.	
	4	The surveillance tools are available and have been disseminated at the subnational level and among some of the designated HFs with identified need for external technical, financial support.	
	5	The surveillance tools are available and have been disseminated at the subnational level and among all of the designated HFs without need for external support.	
6.4. Health and care workers at designated HFs have been trained on the case definition of the priority public health threat and know the reporting mechanism for infected cases.	1	No training on case definition or on the reporting system has been conducted for health and care workers in the designated HFs.	
	2	The training on case definition and the reporting system is still under development or scheduled but has not yet started.	

Sub-capabilities	Scores		Resources and tools:
	3	Health and care workers from the designated HFs have been trained at the national level (for example, through train the trainers programmes), but the training has not yet cascaded to the health and care workers at other levels.	
	4	Health and care workers in some of the designated HFs have been trained on the case definition and reporting system.	
	5	Health and care workers in all of the designated HFs have been trained on the case definition and reporting system.	
6.5. There are functional laboratory services in place supporting the surveillance system with identification of the threat pathogen(s) at the national and subnational levels and among the designated HFs.	1	No laboratory services are available.	
	2	The laboratory services are under development.	
	3	Laboratory services are available only at the national (reference laboratory) level.	
	4	The laboratory services are available at the national and subnational levels and among some of the designated HFs, with need for external support (e.g. samples must be sent abroad, technology is absent, health and care workers are not fully trained).	
	5	The laboratory services are available the national and subnational levels and among all of the designated health facilities and the country can manage them without external support (e.g. technology, financial, supplies and logistics).	

Improvement plan

Improvement Plan							
#	CAPE	Identified Gaps	Anticipatory Actions	Responsible persons	Timeframe	Budget needed	Progress
1	National surveillance and reporting programme for early detection of the priority public health threat(s), especially among hospitalized patients and health and care workers.						
2	IPC and WASH readiness plans needed for the priority public health threat(s) are in place.						
3	IPC and WASH guidelines, SOPs and/or standards for priority public health threat(s) are in place at the national level and in the designated priority HFs.						
4	IPC and WASH training programmes on the priority public health threat(s) are in place at the national, subnational and designated HF(s) levels.						
5	IPC and WASH assessments at the designated HFs to evaluate the readiness to respond to priority public health threat(s).						
6	National surveillance and reporting programme for early detection of the priority public health threat(s), especially among hospitalized patients and health and care workers.						

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Annex 1. IPC and WASH HERO CAPE checklist

Number		Capabilities	CAPE score	Comments\Notes	Verifiers
1		IPC and WASH **coordination mechanism**(s) is/ are in place at the national, subnational and designated priority HF levels.	0%		
1.1	Sub-capabilities	There is a coordination mechanism in place that provides oversight of the IPC and WASH strategy, technical and operational actions and activities for the priority public health threats (e.g. national IPC committee, IPC task force or technical working groups (TWGs)) supported (e.g. co-chaired) by WHO and including all relevant stakeholders and implementing partners, such as national/ international NGOs and health-care facilities' leaders.			• copy of the IPC coordination mechanism TORs • copy of minutes from coordination meeting(s • copy of the partners list (who, what, where, when and why) • copy of the meeting minutes, if any • link for the platform • copy of provided material • any evidence the dissemination • copy of the IPC stakeholders list • copy of the meeting minutes, if any
1.2		There is a communication strategy in place that has been adapted from and integrated with the broader, existing one, to disseminate IPC- and WASH-related information on the priority public health threat or outbreak to the stakeholders and various groups (HFs, health and care workers, community and public social media) using available platforms (social media, official website, WhatsApp groups, contact list emails, hotlines, etc.)			

Number		Capabilities	CAPE score	Comments\Notes	Verifiers
		in coordination with partners in risk communication and community engagement (RCCE).			
1.3		There is an IPC partners' mapping mechanism in place using the "5 Ws" to prepare and coordinate the IPC readiness activities for the priority public health threat(s).			
1.4	**Sub-capabilities**	There is a functioning IPC stakeholder network in place at the national and subnational levels and among the designated HFs to handle priority public health threat(s). For example, IPC national focal points (FPs), quality and patient safety FPs, IPC leaders at the health facility level, hospital administrators and key decision-makers at the national and subnational levels for readiness and response to the identified priority public health threat(s).			
2		IPC and WASH **readiness plans** needed for the priority public health threat(s) are in place.	**0%**		
2.1	**Sub-capabilities**	There is a national contingency plan for specific priority public health threat(s) that includes IPC and WASH readiness activities with an allocated budget.			• copy of the plan(s) • list of the designated priority HFs • copy of the referral system
2.2		There is a surge-capacity plan for the priority threat with allocated resources for IPC and WASH			

Number		Capabilities	CAPE score	Comments\Notes	Verifiers
		activities/ interventions, including staffing, infrastructure and supplies.			
2.3	**Sub-capabilities**	There is a responsive stockpiling process in place to ensure adequate IPC and WASH supplies and equipment.			
2.4		There is a mechanism in place to identify the priority HFs ensuring the coordination, delivery of resources (e.g. supplies) and an isolation-referral system in the event of a surge.			
3		IPC and WASH **guidelines, policies, SOPs and/ or standards** for priority public health threat(s) are in place at the national level and in the designated priority HFs.	**0%**		
3.1	**Sub-capabilities**	There are national IPC guidelines, policies and/or SOPs for the priority public health threat(s).			• copy of/link for the guidelines, policies and/or SOPs
3.2		There are national WASH standards for the priority public health threat(s).			
3.3		There is a dissemination plan for the national IPC and WASH guidelines, policies, SOPs and/ or standards on the priority public health threat(s) on the national/subnational levels (e.g. MOH, National-level CDC) to the designated HF(s).			

Number		Capabilities	CAPE score	Comments\Notes	Verifiers
4		IPC and WASH **training programmes** on the priority public health threat(s) are in place at the national, subnational and designated HF levels.	0%		
4.1	Sub-capabilities	The IPC and WASH training curriculum and materials on priority public health threat(s) have been developed or adapted from the national guidelines.			• copy or link of the training materials • copy of list of the trained staff or certificates • copy of the training schedule • link of the IEC materials • video or pictures of the simulation exercise
4.2		There is a national and/or subnational IPC and WASH training programme that actively provides training on priority public health threats.			
4.3		There are IPC and WASH information, education and communication (IEC) materials developed that communicate relevant messages for health and care workers, patients, visitors and the general population on the identified priority public health threat(s) and that have been disseminated to the designated HFs and to the community.			
5		IPC and WASH **assessments** at the national, sub-national and the designated HFs to evaluate the readiness to priority public health threat(s).	0%		
5.1	Sub-capabilities	There is a system in place that includes all the resources required (trained staff, tools, logistic and financial support) to conduct the IPC and WASH assessment at the designated HFs.			• copy of the tools • copy of list of the trained staff or certificates • copy of the recent assessments • copy of the action/ improvement plans

Number		Capabilities	CAPE score	Comments\Notes	Verifiers
5.2	Sub-capabilities	IPC and WASH simulation exercises are conducted to test the response to the priority threats or are scheduled to be completed within six months from the time that the priority public health threat(s) has been identified.			
5.3		The IPC assessments have been completed in designated HFs using an IPC-assessment tool appropriate for the priority public health threat(s).			
5.4		The WASH assessments have been completed in designated HFs using a WASH assessment tool appropriate for the priority public health threat(s).			
6		National **surveillance and reporting programme** for early detection of the priority public health threat(s), especially among hospitalized patients and health and care workers.	0%		
6.1	Sub-capabilities	Active coordination with the national and subnational surveillance networks that includes syndromic and microbiologic surveillance for the priority public health threat(s) is in place.			• copy of the surveillance plan • copy of the case definition of the priority public health threats • copy of the surveillance protocol TOR • copy of the surveillance tool(s) • copy of list of the trained staff or certificates • copy of the MOU • list of the laboratories • list of the tests performed for the priority public health pathogen(s)

Number		Capabilities	CAPE score	Comments\Notes	Verifiers
6.2	Sub-capabilities	There is a plan and mechanism in place to implement reporting and data collection on the infected patients and health and care workers that includes the occupational management of health and care worker infections.			
6.3		The surveillance tools for priority public health threats with standardized case definition are in place and have been disseminated to the designated HFs.			
6.4		Health and care workers at designated HFs have been trained on the case definition of the priority public health threat(s) and know the mechanism for reporting infected cases.			
6.5		There are functional laboratory services in place supporting the surveillance system with identification of the threat pathogen(s) at the national and subnational levels and among the designated health facilities.			

Score summary	
Total not ready	0
Total minimally ready	0
Total partially ready	0
Total mostly ready	0
Total completely ready	0
Compliance to coordination mechanism	0%
Compliance to readiness plans	0%
Compliance to guidelines, sops and standards	0%
Compliance to training programmes	0%
Compliance to assessments	0%
Compliance to surveillance and reporting programme	0%
Overall ipc and wash readiness (%)	0%
Overall interpretation	**Not Ready**
Assessment completion	0%

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